

Minor Wound Care

Patient Group Direction, version 003, issued 9 September 2026. Two arms: co-amoxiclav and flucloxacillin

Scope of this PGD

This PGD authorises the supply of an oral antibiotic for an infected minor wound in patients aged 2 years and over, alongside wound care advice and an assessment of tetanus status.

- **Co-amoxiclav is used for infected bite wounds and heavily contaminated wounds. Flucloxacillin is used for infected non-bite wounds. The choice is made on the mechanism of the wound.**
- **Every observation in Appendix 1 must be measured and recorded before any supply. If any threshold is breached, refer.**
- **Tetanus immunisation status must be established and recorded for every patient. See Appendix 1.**
- Both arms are beta-lactams. A penicillin-allergic patient cannot be treated under this PGD and is referred, with the likely alternative named.

Arm 1 , Co-amoxiclav (bites and heavily contaminated wounds)

Patient Group Direction for the supply of co-amoxiclav 500/125mg tablets for infected bite wounds and heavily contaminated wounds in patients aged 12 years and over.

Choose this arm where the wound is a bite, or is heavily contaminated with soil or organic material. Pasteurella and Eikenella are not reliably covered by flucloxacillin.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. Neither medicine is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to wound assessment, documented and overseen by the Get Real Health team.
- Must be competent in measuring and recording the observations in Appendix 1, and in recognising the necrotising fasciitis features listed there.
- Must be able to assess tetanus immunisation status and apply the referral rule below.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Infected animal or human bite wounds, and infected heavily contaminated wounds, in patients aged 12 years and over.
Inclusion criteria	• Aged 12 years and over. • A bite wound, or a heavily contaminated wound, with signs of infection: erythema, warmth, swelling, tenderness or purulent discharge. • The bite is NOT to the face or hand. • All observations within the thresholds in Appendix 1. • Tetanus immunisation status established and up to date, or the patient referred for it separately. • Valid informed consent given. • No exclusion criterion present.
Exclusion criteria	Refer, do not supply, where any of the following is present. • ANY feature suggesting necrotising fasciitis: pain out of proportion to appearance, rapidly advancing erythema, crepitus, skin necrosis, bullae or dusky discoloration. Emergency assessment. • Any observation outside the thresholds in Appendix 1, or any sign of systemic illness or sepsis. • Tetanus-prone wound where the immunisation history is incomplete, unknown, or where human tetanus immunoglobulin may be indicated. Refer the same day; HTIG is not covered by this PGD. • Wound requiring closure, exploration or surgical review, or a retained foreign body. • Deep or penetrating wound, or any wound over a joint, tendon or bone. • Bite to the face or hand. • Abscess requiring drainage. • Immunosuppression of any kind, or diabetic foot wound. • An antibiotic already taken for this episode. • Known penicillin or cephalosporin allergy. Refer; see the note on alternatives below. • History of co-amoxiclav-associated jaundice or hepatic dysfunction.

	• Infectious mononucleosis or acute lymphoblastic leukaemia. • Severe hepatic dysfunction, or eGFR below 30. • Under 12 years of age.
Cautions	• Co-amoxiclav may be supplied in pregnancy and breastfeeding where clinically indicated. • Possible increased anticoagulant effect; where the patient is anticoagulated, refer rather than supply. • Advise that any rash should stop the course and prompt review.
Actions if the patient is excluded or declines	Explain why treatment cannot be supplied and arrange the appropriate assessment. Where any necrotising fasciitis feature is present, arrange EMERGENCY assessment. FOR PENICILLIN ALLERGY: both arms of this PGD are beta-lactams, so a penicillin-allergic patient cannot be treated here. Refer to the GP and tell them what is likely to be needed, rather than saying only that alternatives exist. For an infected bite the usual choice is doxycycline with metronidazole; for a non-bite wound infection, clarithromycin or doxycycline. Name this in the referral so the patient is not sent away empty-handed. FOR TETANUS: where the wound is tetanus-prone and the history is incomplete or unknown, refer the same day. Human tetanus immunoglobulin is not covered by this or any GRH PGD. Where only a booster is needed and the patient is otherwise well, the Tetanus (Td/IPV) PGD may be used. Document the advice and the decision, and inform the GP where the reason for exclusion is a new clinical finding.

The medicine

Name, form and strength	Co-amoxiclav 500/125mg tablets.
Legal category	POM.
Route and method	Oral. Take at the start of a meal to reduce gastrointestinal upset.
Dose and frequency	One tablet (500/125mg) three times a day for 5 to 7 days.
Quantity to be supplied	15 tablets for a 5 day course, or 21 tablets for a 7 day course.
Maximum treatment period	7 days. One course per episode. A second course is not authorised; refer.
Adverse effects	Very common: diarrhoea, nausea. Common: rash, vomiting, candidiasis. Uncommon: abdominal pain, indigestion, headache. Rare but serious: anaphylaxis, Stevens-Johnson syndrome, toxic epidermal necrolysis, cholestatic hepatitis, C. difficile infection.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • The observations from Appendix 1. • The mechanism, site and extent of the wound, and whether it was a bite. • Tetanus immunisation status and the action taken. • Which antibiotic was chosen and why. • Name and registration number of the healthcare professional supplying.

	• Name of the medicine, date of supply, dose, form, route and quantity supplied. • Batch number and expiry date. • Advice given, including advice given if excluded or declining treatment. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under 18s: until the 25th birthday.
Storage	Store below 25C.

Information for the patient

Written information	Supply the co-amoxiclav patient information leaflet.
Counselling	• Take one tablet three times a day at the start of a meal, and finish the course. • Seek help THE SAME DAY if the pain becomes severe or out of proportion to how the wound looks, if redness spreads quickly, if the skin darkens or blisters, or if you develop fever, shivering or feel generally unwell. • Seek help if you see red streaks tracking away from the wound. • Stop and seek urgent help if you develop a rash, wheeze, or swelling of the lips or tongue. • Report yellowing of the eyes or skin, or dark urine, even after finishing. • Keep the wound clean and dry. Come back if it is no better in 2 to 3 days.
Follow-up	Review in 2 to 3 days if not improving, and sooner if any warning sign develops. Ensure the tetanus question has been answered before the patient leaves.

Arm 2 , Flucloxacillin (non-bite wound infection)

Patient Group Direction for the supply of flucloxacillin for infected non-bite wounds and localised skin infection in patients aged 2 years and over.

Choose this arm for non-bite wound infection where staphylococcal or streptococcal infection is likely. Do NOT use it for bite wounds; use Arm 1.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. Neither medicine is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to wound assessment, documented and overseen by the Get Real Health team.
- Must be competent in measuring and recording the observations in Appendix 1, and in recognising the necrotising fasciitis features listed there.
- Must be able to assess tetanus immunisation status and apply the referral rule below.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Infected non-bite wounds and localised skin and soft tissue infection in patients aged 2 years and over.
Inclusion criteria	• Aged 2 years and over. • A NON-BITE wound with signs of infection. • All observations within the thresholds in Appendix 1. • Tetanus immunisation status established and up to date, or the patient referred for it separately. • Valid informed consent given (from a person with parental responsibility where the patient is a child). • No exclusion criterion present.
Exclusion criteria	Refer, do not supply, where any of the following is present. • ANY feature suggesting necrotising fasciitis: pain out of proportion to appearance, rapidly advancing erythema, crepitus, skin necrosis, bullae or dusky discoloration. Emergency assessment. • Any observation outside the thresholds in Appendix 1, or any sign of systemic illness or sepsis. • Tetanus-prone wound where the immunisation history is incomplete, unknown, or where human tetanus immunoglobulin may be indicated. Refer the same day; HTIG is not covered by this PGD. • Wound requiring closure, exploration or surgical review, or a retained foreign body. • Deep or penetrating wound, or any wound over a joint, tendon or bone. • Bite to the face or hand. • Abscess requiring drainage. • Immunosuppression of any kind, or diabetic foot wound. • An antibiotic already taken for this episode. • Any bite wound, animal or human. Flucloxacillin does not reliably cover <i>Pasteurella multocida</i> or <i>Eikenella</i>; use Arm 1. • Known penicillin or beta-lactam allergy. Refer; see the note on alternatives in Arm 1. • History of flucloxacillin-associated jaundice or hepatic dysfunction.

	• Under 2 years of age. • Severe renal impairment (creatinine clearance below 10 mL/min).
Cautions	• Flucloxacillin may be supplied in pregnancy and breastfeeding where clinically indicated. This aligns with the Skin and Soft Tissue Infection PGD. • Hepatic reactions may occur up to two months after treatment; advise the patient to report jaundice or dark urine.
Actions if the patient is excluded or declines	As Arm 1, including the named alternatives for penicillin allergy and the tetanus referral route.

The medicine

Name, form and strength	Flucloxacillin 500mg capsules; flucloxacillin 250mg/5mL oral suspension for children unable to swallow capsules.
Legal category	POM.
Route and method	Oral. Take on an empty stomach, one hour before or two hours after food.
Dose and frequency	Adults and children 10 years and over: 500mg four times daily. Children 2 to 9 years: 250mg four times daily, which is 5 mL of the 250mg/5mL suspension four times daily. CHECK THE VOLUME AGAINST THE STRENGTH BEFORE SUPPLY: the 250mg/5mL suspension delivers 50mg per mL. Version 002 stated 10 mL against a 250mg dose, which is double. Duration 5 to 7 days.
Quantity to be supplied	Capsules: 20 for a 5 day course, 28 for a 7 day course. Suspension: 100mL for 5 days, 140mL for 7 days.
Maximum treatment period	7 days. One course per episode. A second course is not authorised; refer.
Adverse effects	Very common: nausea, diarrhoea. Common: rash, vomiting, abdominal discomfort. Rare but serious: anaphylaxis, cholestatic jaundice and hepatitis (may be delayed up to two months), C. difficile infection.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • The observations from Appendix 1. • The mechanism, site and extent of the wound, and whether it was a bite. • Tetanus immunisation status and the action taken. • Which antibiotic was chosen and why. • Name and registration number of the healthcare professional supplying. • Name of the medicine, date of supply, dose, form, route and quantity supplied. • Batch number and expiry date. • Advice given, including advice given if excluded or declining treatment. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Adults: 8 years. Under 18s: until the 25th birthday.

Storage	Capsules below 25C. Reconstituted suspension: refrigerate and discard after 7 days.
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Information for the patient

Written information	Supply the flucloxacillin patient information leaflet.
Counselling	• Take on an empty stomach, an hour before food or two hours after, and finish the course. • Seek help THE SAME DAY if pain becomes severe or out of proportion, redness spreads quickly, the skin darkens or blisters, or you develop fever or feel generally unwell. • Seek help if you see red streaks tracking away from the wound. • Stop and seek urgent help if you develop a rash, wheeze, or swelling of the lips or tongue. • Report yellowing of the eyes or skin, or dark urine, even weeks after finishing. • Keep the wound clean and dry. Come back if it is no better in 2 to 3 days.
Follow-up	As Arm 1.

Appendix 1 , Observations, red flags and tetanus

Every observation must be measured and recorded before any supply. If ANY threshold is breached, do not supply: refer. These thresholds match the Skin and Soft Tissue Infection and Acute Bacterial Bronchitis PGDs.

Temperature	• REFER if 38C or above.
Pulse	• REFER if above 110 at rest.
Respiratory rate	• REFER if 22 or above.
Blood pressure	• REFER if systolic below 100.
Oxygen saturation	• REFER if SpO2 below 94% on air at rest.
Level of consciousness	• REFER on any new confusion or drowsiness.

Necrotising fasciitis , emergency, not a referral

Any one of these means emergency assessment now.

- Pain out of proportion to the appearance of the wound.
- Rapidly advancing erythema, spreading over hours.
- Crepitus, or gas in the tissues.
- Skin necrosis, bullae, or dusky discoloration.
- Anaesthesia over the affected skin.
- Systemic illness out of keeping with the wound.

Other referral triggers

- Lymphangitis: red streaks tracking proximally from the wound.
- Spreading cellulitis, or erythema extending well beyond the wound margin.
- Deep or penetrating wound, retained foreign body, or a wound needing closure.
- Wound over a joint, tendon or bone.
- Bite to the face or hand.
- Immunosuppression, or diabetic foot wound.

Tetanus

- Establish and record the tetanus immunisation history for every patient.
- **A tetanus-prone wound with an incomplete or unknown history: refer the same day. Human tetanus immunoglobulin may be needed and is not covered by any GRH PGD.**
- A tetanus-prone wound in a fully immunised patient needing only a booster: the Tetanus (Td/IPV) PGD may be used.
- Tetanus-prone wounds include puncture wounds, wounds contaminated with soil or manure, wounds with devitalised tissue, and wounds presenting after 6 hours.

Authorisation

Version	v003
Supersedes	Version 002, 7 September 2026
Valid from date	9 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signed: N. Shori Date: 9 September 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signed: C. Pilkington Date: 9 September 2026

Both authorising signatories reviewed this version together on 9 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name: Job title and organisation: Signature: Date:

Change history

Version	v003
Date	9 September 2026
Changes	Paediatric flucloxacillin volume corrected. Version 002 read "Children 2 to 9 years: 250mg four times daily (10mL of the 250mg/5mL suspension)". 250mg of that suspension is 5 mL. The quantity box, 100mL for 5 days and 140mL for 7 days, was calculated correctly for 5 mL, so the parenthetical was the error and a child would have received double the intended dose. The identical error was found and corrected in the Skin and Soft Tissue Infection PGD on the same day; it had been copied between the two documents. Found by an automated internal consistency scan that recomputes every stated dose volume against its strength, not by anyone reading it. This is a targeted dose correction only: this document has not had a full clinical review and one remains outstanding.

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Premises name
 Address
 Postcode:
 ODS code, if applicable

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

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